# TE-07 Overdose / Toxic Ingestion

**CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026**

**Ventilation and perfusion come first. Naloxone is titrated to adequate breathing, not return of consciousness. Use Poison Help early and do not import legacy source-PDF antidote doses.**

## Recognition and immediate field flow

| Step | Approved action |
|---|---|
| **1. ABCs and history** | Support airway, breathing, circulation, glucose, and temperature. Identify substance, route, amount, time, co-ingestants, intent, containers, and exposure mechanism when safe. |
| **2. Inadequate ventilation** | Ventilate first. For suspected opioid-induced respiratory depression, administer naloxone only per the current Claiborne Medication Reference. Do not delay airway or ventilation care. |
| **3. Shock, AMS, seizure** | Treat under the age-appropriate shock, diabetic, altered-mental-status, behavioral, and seizure pathways. |
| **4. Poison Help** | Call 1-800-222-1222. Obtain 12-lead, IV/IO, and cardiac monitoring as indicated. Use Medical Control for severe exposure or care outside standing orders. |
| **5. Specific toxin** | Consider APAP/salicylate, beta/CCB, CO/cyanide, organophosphate, TCA, and other toxidromes. Use the current Medication Reference and Poison Help/Medical Control; do not import legacy antidote doses. |
| **6. Disposition** | Patients treated with naloxone require healthcare observation for recurrent respiratory depression. Other low-risk exposures may follow Poison Help home-care advice only when agency non-transport criteria are met. |

## Provider-level actions

| Level | Approved actions |
|---|---|
| **EMT** | Support ventilation; administer naloxone for suspected opioid-induced respiratory depression only per the current Medication Reference; check glucose; contact Poison Help; and support decontamination when indicated. |
| **AEMT** | All EMT care plus IV/IO access, broader-route naloxone per the current Medication Reference, reassessed fluids, and toxin-specific treatment only with Poison Help or Medical Control guidance. |
| **Paramedic** | All prior care plus cardiac monitoring, pacing when indicated, and toxidrome-specific advanced care using the current Medication Reference with Poison Help or Medical Control guidance. |

## Critical safeguards

- Charcoal is not routine. Use only when stocked, authorized, recommended by Poison Help or Medical Control, and when airway protection and swallowing are intact.

- Do not clear a patient only because they awaken after naloxone; recurrent respiratory depression can occur and requires healthcare observation.

- For skin/eye exposures, remove contaminated clothing and irrigate. For inhalation, move to fresh air only when the scene is safe.

## TE-07 | TOXIDROME SAFEGUARDS, CONSULTATION, AND HANDOFF

### Opioid and ingestion safeguards

| Topic | Required action |
|---|---|
| **Opioid exposure** | Open the airway and provide breaths or BVM ventilation for respiratory depression or arrest with a pulse. Naloxone does not replace ventilation and must not interrupt high-quality CPR when cardiac arrest is present. |
| **Naloxone response** | A recurrent decline in mental status or breathing can occur after initial response. Continue monitoring and arrange healthcare observation; repeat treatment follows the current Medication Reference and clinical reassessment. |
| **Intentional ingestion** | Perform a focused safety assessment for hidden medications and weapons. Intentional, pediatric, pregnant, polysubstance, or symptomatic exposures require direct Poison Help consultation. |
| **Nerve-agent concern** | For suspected organophosphate or nerve-agent exposure, use TE-08 for responder safety, decontamination, cholinergic recognition, and escalation. Claiborne ground units do not carry a CHEMPACK/Mark I field kit. |

### Consultation, destination, and handoff

- Report substance/product, route, amount, timing, co-ingestants, intent, toxidrome, ECG findings, glucose, decontamination, airway/ventilatory support, treatment, response, and Poison Help/Medical Control guidance.

- Use the applicable airway, shock, seizure, cardiac-arrest, behavioral, diabetic, CO/cyanide, and chemical/HazMat pathways as indicated.

- The current Claiborne Medication Reference controls medication selection, route, dose, contraindications, and monitoring for agent-specific treatment.

**Critical reminder: Poison Help is 1-800-222-1222. Early consultation, effective ventilation, safe decontamination, and transport/observation when indicated take priority over legacy dose tables or unstocked antidote searches.**